



GUIDELINE

Shared Care in CAMHS

Scope (Staff):	All clinical staff
Scope (Area):	CAMHS

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

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Aim

To guide CAMHS clinical staff in delivering safe, effective shared care to children and young people.

Risk

Safety and quality of care for children and families are not maintained during period/s of shared care or transitions in care.

Background

Shared care is defined as systemic and operational cooperation, where clinically appropriate, between two or more health professionals and / or services, with a view to providing optimal, high quality care. Other terms used in literature include collaborative care, split care and combined care.¹

In CAMHS, the involvement of multiple services in a child's care does not necessarily invoke shared care; shared care only occurs when there is a sharing of the assessment, treatment, and management of the child. Shared care does not necessarily involve the sharing of information between services; this should only occur where clinically relevant and in accordance with the [CAMHS Consent Guideline](#) (see Consent to Exchange of Information) and [CAHS Confidentiality, Disclosure and Transmission of Health Information Policy](#)

This guideline applies to both children who are being cared for by two or more CAMHS teams, and children who are being cared for by one CAMHS team and one or more external health professionals:

- When the child is being cared for by two or more CAMHS teams, the principles outlined in this guideline must be upheld.
- When the child is being cared for by one CAMHS team and one or more external health professionals, CAMHS staff must make every effort to engage the external party in a shared care agreement (SCA) that reflects the principles of shared care as they are outlined in this guideline.

This Shared Care Guideline should be read in conjunction with the [CAMHS Clinical Handover Guideline](#) to ensure shared care arrangements are effectively communicated thereby facilitating the delivery of comprehensive care and minimising clinical risk.

Definitions

Child: For the purposes of this guideline, an infant, child, adolescent or young person under the age of 18 years.

Family: any parent, legal guardian or identified carer of the child.

External health professional / service: For the purposes of this guideline, external refers to any health professional / service outside of CAMHS. This includes primary care (e.g. General Practitioners), non-government organisations, private sector, other Department of Health services (e.g. Youth, Child Development Service, Adult Mental Health), and other government services (e.g. Department of Communities, Department of Education).

Medical record: A medical record is the compilation of health information pertaining to the health care provided to a patient of the health service. Medical records are inclusive of paper-based records and electronic records (digital medical records).

Principles / Key points

- Consumers have the right to comprehensive and integrated mental health care that meets their individual needs and achieves the best possible outcome in terms of their recovery (Criteria 2.1, [Chief Psychiatrist's Standards for Clinical Care](#)).
- Recovery oriented practice provides a holistic framework that informs all contact with children and their families. It builds and enhances strength, resilience, and social and emotional wellbeing. It supports children to return to normal developmental pathways. It is underpinned by the premise that children do recover from mental health problems. It engages all areas of the child and family's life, including relationships, education, vocation, and leisure.
- Children and their families are part of a wider community, and mental health services are one part of a wider service network. CAMHS supports the provision of coordinated and integrated care across programs sites and services.
- When shared care is provided, all CAMHS clinicians must commit to:
 - timely and collaborative communication;
 - effective risk management; and
 - high quality clinical care, having regard to the Charter of Mental Health Care principles as per the [Mental Health Act 2014 \(WA\)](#).
- Clear collaborative identification of roles and responsibilities around risk management will optimise outcomes for the child. Identification and reduction of risk within the SCA must occur as outlined in the [CAMHS Risk Assessment and Management Policy](#).
- Any issues relating to the use of Psychiatric Services Online Information System (PSOLIS) should not be considered a barrier to effective shared care. The key principles of effective communication, risk management and high-quality clinical care are not dependent on the functionality of PSOLIS.

Also refer to:

- [CAHS Vision and Values](#)
- [CAHS Speaking up for Safety program](#)
- [CAMHS Risk Assessment and Management Policy](#)
- For legal issues related to capacity to consent: [Mental Health Act 2014](#) and [CAMHS Assessing Capacity to Consent in Minors a clinical handbook](#).

Process

See flowchart in Appendix 1

Note: Shared care should only proceed where the involved parties agree that the potential benefits of shared care **are not** outweighed by the risks. In the following situations, the potential benefits of shared care may be outweighed by the risks:

- When the child and family are not engaged in the process;
- When there is a history of the family seeking different services with evidence of lack of positive engagement with any service provider;
- When there is no agreed formulation between the parties engaged in the shared care process;
- When a SCA has been demonstrated to increase risk; or
- Where there is no capacity for regular liaison between services.

Steps	Details	Documentation
1. Request for shared care	Requesting service/professional sends a written referral / request for shared care; OR Child / family makes a request to initiate shared care.	
2. Discussion with the requesting service / professional	CAMHS service assesses the request and makes contact with the relevant service/professional. This should include discussion of: • the child's suitability for the CAMHS service; • expectations of the requesting service/professional; and • current service capacity (e.g. expected wait time for entry into the service).	Document discussion in the medical record. If applicable, update PSOLIS/webPSOLIS with this information.
3. Discussion with the child and family	The child and family must be engaged in collaborative decision making regarding the decision to enter into a SCA. The child and family	Document decision in the medical record. If applicable, update PSOLIS with this information.

Steps	Details	Documentation
	must consent to the decision to enter into a SCA, with each child's capacity to consent assessed in keeping with the principles of the Mental Health Act 2014. Where appropriate, legal guardian must be ascertained. Note that the absence of a SCA will not affect the child's eligibility to access CAMHS services.	Provide the FAQ: Shared Care at CAMHS resource to the child and family
4. Discussion with the Multidisciplinary Team (MDT)	The MDT must reach a collaborative decision as to whether shared care is in the best interest of the child in the context of the clinical situation (refer to note above regarding situations in which the potential benefits of Shared Care may be outweighed by the risks).	Document decision in the medical record and on PSOLIS.
5. Child, family and MDT agree that shared care may not be in the best interest of the child	Collaborative decision making must be informed by: • close communication between clinicians; • outcome of assessment, including comprehensive risk assessment; • exploration and discussion of all available options; • best-practice service delivery; and • discussion and consideration of the risks and benefits of shared care, with professional opinions conveyed in an open and honest manner.	Document decision, including rationale, in the medical record and provide the child and family with a written copy. The documentation must include details of the lead agency/ professional who will be offering ongoing care. If applicable, update PSOLIS with this information.

Steps	Details	Documentation
6. Child, family and MDT agree that shared care <u>is</u> in the best interest of the child	A written SCA must be developed. - For SCAs between two CAMHS teams, the <u>CAMHS SCA form</u> must be used to document the SCA. - For SCAs between a CAMHS team and an external agency, the use of the <u>CAMHS SCA form</u> should be considered. - Where the external party does not wish to utilise the CAMHS SCA form, the CAMHS clinician/s should aim to ensure that the written alternative reflects the information contained in the CAMHS SCA form. The child/family should be included in the process of developing the written SCA as this forms the basis of ongoing management. The written SCA must include: - Roles and responsibilities of all parties, including the specific treatment and care to be provided by each involved professional; - Parental and carer views and child's wishes and best interest; - Plans towards recovery;	File the written SCA in the medical record. Update the management plan and, if required, the risk assessment management plan (RAMP) and crisis management plan. Ensure that all information relevant to the SCA is recorded on PSOLIS.

Steps	Details	Documentation
	• Clear timelines for reviews; • Roles and responsibilities for emergency assessment and management both within and outside working hours; • Agreed method and frequency of communication between involved professionals and the child and family; • Mechanism for instigating or increasing the frequency of communication according to clinical need (for example, at times of heightened clinical risk). Any significant change in clinical presentation must act as a trigger to instigate communication. Examples include, but are not limited to, change of medication, change in mental state, change in risk or change in concordance with the agreed management plan; and • Plan for continuation / transfer of care should the child become ineligible for one or more of the services involved in the shared care arrangement (for example, due to their age).	
7. Review of SCA	A decision to review the SCA may be made for a	

Steps	Details	Documentation
	number of reasons, including but not limited to: • A request by one of the parties involved in the SCA (including the child/family); • An increase in clinical need or risk; • A change in Case Manager; or • A decision – by either of the services involved in the SCA – to discharge the child. If a decision is made to review the SCA, the involved parties must meet (ideally face-to-face) to clarify points of concern. They may choose to either: • Problem solve, resolve difficulties and amend the SCA, OR • Plan toward termination of shared care, with a clear understanding of the professional/service that will have primary ongoing responsibility for care.	
8. Change to the SCA	Any change to the SCA requires informed consent of all parties including the child and family. Original signatures are not mandatory as long as agreement/consent can be evidenced (e.g. by email, for instance).	Update SCA, management plan, and, if required, the RAMP and crisis management plan. Ensure that all information relevant to the SCA is recorded on PSOLIS.
9. Termination of SCA	Shared care may be reinstated at a later point in time if clinically appropriate.	If child is discharged from CAMHS, complete care transfer summary / discharge documents.

Issue resolution

Any disputes arising out of the above process must be dealt with using the existing clinical governance and escalation process in each service. Within CAMHS, any issues that cannot be resolved at service-level (e.g. via the MDT meeting) must be escalated to the next level of line management, such as the Service Manager and Head of Service, or Director and Head of Department.

Professional considerations for CAMHS clinicians

- CAMHS clinicians should be respectful of others' roles and expertise and be mindful of not undermining one another, as even implied criticism can hamper the success of the SCA. However, if a CAMHS clinician holds concerns regarding the training, experience or credentials of any of the professionals involved in the SCA, it would be unethical to participate in the process. In such a case, the CAMHS clinician should raise these concerns with their line manager.
- CAMHS clinicians must adhere to principles of good clinical practice and their respective professional and ethical standards.
- When there are differences of opinion, every attempt must be made to resolve these in an open environment, preferably by organising a case conference which also includes the child and family.
- CAMHS clinicians involved in shared care must demonstrate full commitment to collaboration and working towards a common goal.

Models of Shared Care

The Integrated Working Model of shared care (sometimes referred to as 'ongoing collaboration') is the most common model used within CAMHS. This model involves an ongoing shared responsibility for the care of the consumer² and aims to create 'seamless patient pathways through the health system.'³ This model is also considered appropriate for sharing of physical health care between agencies.

If a CAMHS MDT makes an assessment that the Integrated Working Model of shared care is not the most appropriate model of shared care for a child and family, consideration may be given to utilising other accepted or locally developed models of shared care that comply with the principles outlined in this guideline. For more information on models of shared care, see Lester et al.⁴

Monitoring compliance

Compliance with this guideline will be monitored at a local level by the CAMHS safety and quality team.

Training

Clinicians must complete the Clinical Skills Training package as part of their mandatory training as employees of CAMHS.

Related external legislation, policies, and guidelines
<u>Mental Health Act 2014 (WA).</u>
Australian Commission for Safety and Quality in Healthcare. National Safety and Quality Health Service Standards. Standard 6: Communicating for Safety. Accessed from: <u>Communicating for Safety Standard Australian Commission on Safety and Quality in Health Care</u>
WA DoH Mandatory Policy: <u>MP 0181/24 Safety Planning for Mental Health Consumers Policy</u>
WA DoH Mandatory Policy: <u>MP 0155/21 State-wide Standardised Clinical Documentation for Mental Health Services Policy</u>
WA DoH Mandatory Policy: <u>MP 0122/19 Clinical Incident Management Policy</u>
Office of the Chief Psychiatrist (WA): <u>Chief Psychiatrist's Standards for Clinical Care</u>
Office of the Chief Psychiatrist (WA): <u>Policy for Reporting Notifiable Incidents to the Chief Psychiatrist – Public Mental Health Services</u>

References
Goldberg RS, Riba M, Tasman A. Psychiatrist's attitudes towards prescribing medication for patients treated by non medical psychotherapists. Hosp Community Psychiatry. 1991; 42:27-280.
Meyer D. Split Treatment and Coordinated Care with Multiple Mental Health Clinicians: Clinical and Risk Management Issues. Primary Psychiatry, 2002. Accessed at <u>http://primarypsychiatry.com/split-treatment-and-coordinated-care-with-multiple-mental-health-clinicians-clinical-and-risk-management-issues/</u> on 14 January 2015.
Lester H. Shared care for people with mental illness: a GP's perspective. Advances in Psychiatric Treatment. 2005 11:133-141
Lester, H., Glasby, J. & Tylee, A. (2004a) Integrated primary mental health care: threat or opportunity in the new NHS? British Journal of General Practice, 54, 285–291.
The <u>Vancouver style</u> referencing is as per CAHS Library and Information Service.

Related CAHS internal policies, procedures and guidelines <i>(if required)</i>
<u>CAMHS Consent Guideline</u>
<u>CAMHS Clinical Handover Guideline</u>
<u>CAMHS Multidisciplinary Team Clinical Reviews Policy</u>
<u>CAMHS Risk Assessment and Management Policy</u>
<u>CAHS Patient/Client Identification Policy</u>
<u>CAHS Confidentiality, Disclosure and Transmission of Health Information Policy</u>
Useful resources (including related forms) <i>(if required)</i>
Shared Care Agreement form - Available to order from PMG, MR510.04
<u>Frequently Asked Questions: Shared Care at CAMHS</u>
<u>Shared Care Agreement Process – Touchstone</u>

This document can be made available in alternative formats on request.

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Appendix 1: Shared Care Flowchart

